



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : نجوي فواد بطرس عبدالمسيح

العنوان : ----

الحى / المدينة : العمرانيه

المحافظة : الجيزه

تاريخ تسجيل الإستماره : 23-09-2026

رقم البطاقة : 26701092500663 رقم التليفون : 01122159665

العمر (تاريخ الميلاد) : 09-01-1967 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
التوقيع: (-----)

الإخصائي الاجتماعي
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونك [14157]

قسم العلاج :
الاورام

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: نجوي فواد بطرس عبدالمسيح National ID: 26701092500663	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
 بطاقة تحقيق الشخصية

نجوى
 فؤاد بطرس عبدالمسيح
 ٦ شحاته طنطاوي - العمرانية الغربية
 العمرانية - الجيزة

١٩٩٧/٠٨/٢٩ ٢٩٧.٠١.٩٢٥.٠٠٦٦٣
 124329425


 ٢٠٢١/٠٨ ٢٩٧.٠١.٩٢٥.٠٠٦٦٣
 حاصله على دبلوم تجاره
 أرملة مسيحية أنثى
 البطاقة سارية حتى ٢٠٢٨/٠٨/٠٢



Patient Name : الأستاذة / منى فؤاد بطرس عبد الحنج
Patient No. : 267010925006
Path No. : 1703/24
Sex : Female
Age : 57 Y
Referring Doc :

Request No. : 13000356
Request Date : 22/05/2024
Verification Date : 26/05/2024
Financial Category : أورام داء السلام - فئة الاولى
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 21/5/2024, 2:00 pm.

Gross Examination:

- Multiple grayish white tissue cores up to 1.2 cm long, totally submitted.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma.
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade II.
- * Ductal carcinoma in situ: Not detected.
- * Perineural invasion: Not detected.
- * Angioinvasion: Not detected.

Diagnosis:

Right breast mass: Tru-cut biopsy
Invasive duct carcinoma, Grade II
For further IHC + ER, PR, Ki67.

Signature
Prof. Dr. Gamal Abd-El-Atty.
Dr. Menna Nabil

Verified by

Printing Date : 26/06/2024 09:24:09

Page 1 of 1

Printing By : marwa mohamed youssef

Patient Name : الد. علاء / ادوى فوزى حسن عبدالرحمن	Request No : 13000356
Patient No. : 267010~2500663	Request Date : 22/05/2024
Path No. : 1703/24	Verification Date : 02/06/2024
Sex : Female	Financial Category : دواء ذات ائتمان - نفقة الزوجة
Age : 57 Y	Department : Out Patient
Referring Doc. :	IHC No
	Registered by : Mrs. Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Right breast mass.

Nature of Specimen: Tru-cut biopsy, collected at 21/04/2024, 2:00 pm.

Technique

Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Observation

- ER: (Positive control showed appropriate reactivity)
- ER: Strong nuclear reaction in 90 % of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90 % of tumor cells (score 8/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67: 10 % nuclear reaction of tumor cells.

Diagnosis:

Right breast mass, Tru-cut biopsy:
invasive duct carcinoma, Grade II.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 10 % of tumor cells.

* Scoring system: "ER, PR"

Verified by

Patient Name	الاستاذ / ا.د.ى فؤاد بطرس عبدالمنعم	Request No.	13000356
Patient No.	26701092500603	Request Date	22/05/2024
Path No.	1703/24	Validation Date	02/06/2024
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	57 Y	Department	Out Patient
Referring Doc.			

Registered by Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense > 10% of tumor cells.

Signature
Prof. Dr. Gamal Abd-El-Aly
Dr. Laila Magdy.

Verified by :

Printing Date : 26/06/2024 09:24:10

Page 2 of 2

Printed By : marwa mohamed youss

CAP

غرفة رقم (4) - الدور الاول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987 - Nile Corniche - Msr El Qadema - Cairo, Egypt

Patient's Name:

NAGWA FOUD BOTROS

Date:

2023-05-01

Referred by:

Prof.Dr.

Hospital:

Conserve Roosseey International Egypt

BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

INDICATION:

Right CBS

TECHNIQUE:

- * Bilateral MLO and CC views
- * Bilateral U/S

REPORT:

- * Dense parenchyma. (ACR C), with focal disruption seen underlying the scar site with mild diffuse skin thickening.
- * No suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- * Complementary U/S showed no solid or cystic mass lesions.
- * Normal skin thickness and nipple-areola complex.
- * Left nonspecific axillary nodes are seen
- * Free right axillary region as well as the infra and supra-clavicular regions.

OPINION:

- * Normal study of the left breast. (BIRADS 1).
- * Right post-operative changes. (BIRADS 2).

Best regards,
Dr. Yara Mohamed, MRD

NAME: nagwa foad botros

DATE: 2/8 /2026

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver:-

- Average in size (15cm at Rt MCL) bright parenchymal echo pattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder:-

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen:-

Average size, homogenous.

Kidneys:-

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder:-

- Not distended enough however no stones or diverticular out pouching

Uterus and adnexal: no abnormalities could be detected

Opinion:

Bright liver likely fatty

Colonic gaseous distention

Best regards

Dr. NERMEEN RAMADAN FEBR, MD

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